Primary Health Care

Goal: To improve the knowledge and skills on primary health care

Specific objectives:

1. To understand the meaning, purpose, origin and levels of primary health care
2. To explain the pillars and elements of primary health care
3. To understand the implementation of primary health care
4. To review the successes and failures of primary health care implementation

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Meaning – Primary Health Care is defined as an essential health care based on practical, scientifically sound, socially acceptable methods, technology made universally acceptable to individuals, families and community through their full participation and affordable cost that the community can sustain and become self-reliant

Purposes– the importance of primary health care are:

- Reach everybody – particularly those in great need
- Reach the home and family levels, and not to be limited to health facilities
- Involve a continuity relationship with persons and families
- Improving family health - with particular focus on mothers and children
- Increase coverage and accessibility of essential health services
- Improve the quality of services
- Pursue an integrated inter-sectorial and multi-disciplinary approaches
- Increase community participation in the planning, delivery, and monitoring of health services

Origin – The concept of PHC started as follows:

- The Declaration of Alma-Ata was adopted in September 1978 at the International Conference on PHC at Almaty (formerly Alma-Ata) in Kazakhstan (formerly Kazakh Soviet Socialist Republic)
- This was the first international declaration underlining the importance of primary health care
- Goals and targets were set for achieving "Health for All" by the Year 2000. Some of these goals were that:
  - ✓ At least 5% of gross national product should be spent on health;
  - ✓ At least 90% of children should have a weight for age that corresponds to the reference values;
  - ✓ Safe water should be available in the home or within 15 minutes' walking distance, and adequate sanitary facilities should be available in the home or immediate vicinity;
  - ✓ People should have access to trained personnel for attending pregnancy and childbirth; and
  - ✓ Child care should be available up to at least one year of age.
- It expressed the need for urgent action by all governments, all health and development workers, and the world community to protect and promote the health of all people
- The PHC approach has since then been accepted by member countries of the WHO as the key to achieving the goal of "Health for All".

Principles – A principle is a rule that has major influence on the way in which something is done. Therefore, the principles of PHC are rules or guidelines that govern the implementation of PHC's activities. The five basic principles are:

- *Equitable distribution*
  - ✓ Equity is the fair distribution of available resources to all individuals and families so that they can meet their fundamental and basic needs.
  - ✓ Services should be physically, socially, and financially accessible to everyone
  - ✓ People with similar needs should have equal access to similar health services
  - ✓ Equal access, resource distribution and coverage of PHC services should be greatest in those areas with the greatest need

- *Manpower development*
  - ✓ PHC aims at mobilizing the human potential of the entire community by making use of available resources
  - ✓ This principle facilitate the identification and development of necessary personnel as well as training new categories of health workers to serve the community

- *Community participation*
  - ✓ This is the process by which individuals, families and communities take responsibility in promoting their own health and welfare
  - ✓ Full participation is important in decision making and taking care of own health
  - ✓ Community members and health providers need to work together in partnership to seek solutions to problems facing the communities
  - ✓ Communities should participate in: Creating and preserving healthy environment; Maintaining preventive and promotive health; Sharing information about own needs with higher authorities; Implementing health care priorities and managing health facilities

- *Appropriate technology*
  - ✓ This is a kind of technology that is scientifically or technically sound and adoptable to local needs, and the community can afford to maintain it for their self-reliance
  - ✓ Caregivers should be trained to deliver services using the most appropriate and cost -effective methods and equipment for their level of care
  - ✓ Full participation is important in decision making and taking care of own health

- *Multi-sector of Inter-sector approach*
  - ✓ Health sector cannot achieve much in isolation
  - ✓ PHC requires coordinated effort with other related sectors whose activities impacts on health e.g., agriculture, water and sanitation, transportation & communication, education etc.
  - ✓ The commitment of all sectors may increase if the purpose for joint actions and

roles of each sector is clearly defined

Levels – A level is a stage or position of something. Levels of PHC are the units of care or implementation of activities. The levels of health care are:

- Individual Based Health Care
- Family Based Health Care
- Community Based health care
- Facility Based Health Care(Dispensary, Health center, District hospital, Provincial hospital)

Pillars – A pillar a column or support. Pillars of PHC are those support systems that make it achieve goal of improved health by creating partnership between consumers of health services with health care providers. The Seven pillars are:

- Health systems
- Priority
- Science
- Culture
- Equity
- Participation
- Sustainability

Elements – Are fundamentals or essentials of PHC. In In the Alma Ata conference of 1978, eight essential elements of PHC were identified. However, individual countries were given the liberty to add any other elements they felt were relevant to their own country. The PHC elements listed at the Alma-Ata declaration were as follows:

1. Health education
2. Local disease control
3. Expanded programme of immunization
4. Maternal and child health care and family planning
5. Essential drug supply
6. Nutrition and adequate food supply
7. Treatment and prevention of common diseases and injuries
8. Safe water supply and good sanitation

Kenya government has added more elements. These are:

1. Mental Health
2. Dental Health
3. Community Based Rehabilitation
4. HIV/AIDS Prevention
5. Health information systems

Strategies – Are approaches or methods of implementing an activity. There are three main PHC strategies:

- *Community Based Health Care*
  - ✓ These are health care activities initiated and implemented by community own

resource persons

- ✓ It is for people of all ages who need health care assistance at home. Community care services include home support, home nursing, physiotherapy and other rehabilitation services.
- ✓ For example: A CHW visits an elderly person at home to help with medication, or a CHW support worker helps a child in a wheel chair with personal care, such as bathing.

- *BAMAKO Initiative*
  - ✓ This initiative was a formal statement adopted by African health ministers in 1987 at Bamako in Mali.
  - ✓ It was meant to implement strategies designed to increase the availability of essential drugs and other health care services for Sub-Saharan Africans
  - ✓ This was an initiative of drug revolving funds as income generating activity to sustain PHC activities.

- *Community strategy*
  - ✓ This is the concept of taking health services close to the people in the village
  - ✓ It is aimed at taking the Kenya Essential Package for Health to the community. It is a strategy for the delivery of level one service
  - ✓ The overall aim is to involve the communities in addressing the downward trend of deteriorating health status. It is a mechanism through which households and communities take an active role in health and health-related development issues.

Roles – Roles are responsibilities assigned to an individual or a group with an aim of achieving a desired objective. The following are key roles of Health Records & Information Officers and Community Health Workers in PHC implementation:

- *Health Records and Information Officer*
  - ✓ Designing, developing and distribution of PHC data collection tools
  - ✓ Data collection, analysis and dissemination
  - ✓ Training of CHW on data collection and management
  - ✓ Advising the PHC team on targets and achievements of PHC
  - ✓ Monitoring and evaluation of PHC implementation

- *Community Health Workers*
  - ✓ Identify and refer patients
  - ✓ Treatment of minor illnesses
  - ✓ Cary out health education
  - ✓ Defaulter tracing
  - ✓ Support home based care services
  - ✓ Home follow up

Success & failures – Are attainment of achievements or inability to attain any intended achievement in any endeavor. The following are reasons why PHC did not achieve its intentions in most of the cross-cutting countries:

- Many ordinary people felt PHC was a cheap form of healthcare and, if they were able to, they bypassed this level to attend secondary and tertiary centers because of a lack of staff and essential medicines at the PHC level.
- Civil war, natural disasters and, more recently, HIV affected the ability of PHC to maintain comprehensive services, especially in many sub-Saharan countries.
- Political commitment was not sustained after the initial euphoria of Alma-Ata. In many cases PHC became a jargon term used as a slogan, and little else. The rhetoric was not backed with the necessary reforms. Agencies were content if countries adopted PHC as a policy, and did not assess actual practice. Politicians saw PHC as a way to reduce expenditure in health and lacked the political will to ensure that services were equitably shared and distributed. Most healthcare resources continue to be directed to the large urban-based hospitals.
- Issues of governance and corruption in the use of resources resulted in donors becoming very wary of funding comprehensive, broad-based programs. Vertical, definable, time-limited programs that could be changed every few years suited both donor agencies and governments.

******** Field Visit to a nearby PHC site *******
Visit a nearby PHC site and identify:

1. **What is the name of the site?**
2. **When did the site start?**
3. **How many CHWs are active?**
4. **What are the PCH services offered?**
5. **What are the 10 factors that are making PHC successful in the site?**
6. **What are the 10 factors that are making PHC fail in the site?**
7. **What are the plans to revitalize PHC activities in the site?**